

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: Wimberley **Period:** December 7 to February 6, 2024 **Report Date:** February 7, 2024 **Coverage:** 1100/1460h (75.3%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 62 days from Dec 07 to Feb 06 ■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
97	114h	Very High Breathing	2	75%

Major Findings: Sustained very high breathing (avg 23, peak 108)

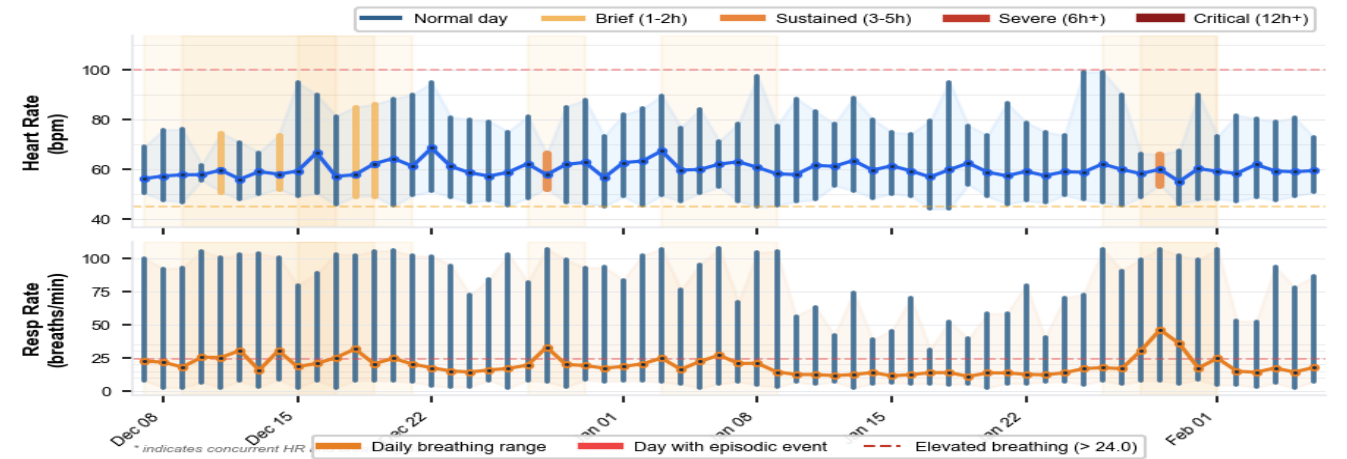
#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/day	Average	Date
1	Very High Breathing	107 brpm	17h	3h	2	27 brpm	Jan 26 to Feb 01
2	Very High Breathing	108 brpm	14h	2h	2	21 brpm	Jan 03 to Jan 09
3	Very High Breathing	106 brpm	29h	2h	2	23 brpm	Dec 07 to Dec 21
4	High Breathing	107 brpm	16h	4h	3	23 brpm	Dec 27 to Dec 30
5	High Breathing	105 brpm	10h	2h	2	31 brpm	Jan 28 to Feb 01
6	High Breathing	103 brpm	17h	2h	2	23 brpm	Dec 09 to Dec 17

+ 1 additional condition(s) across Elevated Breathing; details in monitoring data.

RR values outside physiologic range (6–60 breaths/min).

Clinical Pattern Observations:

- 1. **Breathing variability:** RR spread 31 brpm (10 to 41).
- 2. **Clustered pattern:** Episodic events on 6 of 62 days (9%).
- 3. **Improving trajectory:** 42 → 27 events vs prior period.



Red bars indicate days with detected episodic events.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.